

Supporting Selective Mutism

A BRIDGE guide for the child who speaks freely at home and can't find their voice anywhere else, across every age and stage.

What Selective Mutism Is (and Isn't)

Selective mutism (SM) is an anxiety disorder, not a choice, a phase, or defiance. A child with SM is physically unable to speak in certain settings, most commonly school, even though they may be chatty and entirely comfortable at home. It's estimated to affect somewhere around 1 to 2 per cent of children (Kumpulainen et al., 1998), which makes it far more common than most adults realise, and far less rare than it feels when you're the one trying to support a single, silent child in a room full of talking ones.

It's easy to mistake for something else, so it's worth being precise: SM isn't oppositional behaviour (the child wants to speak and can't, rather than refusing to), isn't autism (children with SM read social and emotional cues typically, especially at home), isn't the result of trauma in most cases, and isn't simply a "silent period" in a second language. Bilingual children can and do develop genuine SM, and deserve a proper assessment rather than an assumption either way (Child Mind Institute, 2022).

Why this guide is organised by age, not by a single script

Selective mutism doesn't look the same in every child, or even in the same child at different ages. A five-year-old freezing during circle time and a fourteen-year-old who's built a years-long habit of silence need genuinely different approaches, not a smaller version of the same one.

The Core Principle: Lower the Pressure, Build the Confidence

Every effective approach to SM shares the same foundation: never coax, never push, and never let a child avoid a situation entirely if you can help it. The goal is small, successful steps toward speech, not forcing speech itself. Pushing a child to do something beyond what they can currently manage tends to backfire, reinforcing the very anxiety you're trying to reduce (Child Mind Institute, 2022).

One trap well-meaning adults fall into constantly: "rescuing" the child by answering for them, or accepting nonverbal communication as the permanent endpoint rather than a bridge. It's kind in the moment and unhelpful over time. It quietly confirms that speech isn't necessary, which is the opposite of what treatment is working toward.

Early support matters. Treatment tends to be more effective the sooner it starts, and outcomes noticeably worsen the longer SM goes unaddressed. By adolescence, only around 30 to 40 per cent of young people who are diagnosed and treated will go on to speak comfortably to a wide circle of peers (Bergman et al., 2002). That's not a reason for alarm; it's the reason this guide exists at every age band, not just the earliest one.

1 Early Years (3–5)

At this age, the goal isn't speech, it's comfort. Nonverbal communication is a legitimate bridge, not a failure.

Accept nonverbal answers: Pointing, nodding, and gestures are valid participation. The goal is to build on them over time, not to withhold engagement until words appear.

Be a sportscaster: Narrate what the child is doing ("you're building a tower") rather than asking questions. It shows interest without demanding a response.

Warm-up time: Where possible, let the child arrive with a parent before the room fills, and practise being in the space without the pressure of an audience.

One trusted peer: A child who's already comfortable with the student is often the easiest first step toward speech, easier than any adult in the room.

2 Primary (6–10)

Kids this age can start working with gentle structure: small, low-stakes wins that build toward bigger ones.

Wait five seconds: After asking something, wait in silence before moving on or answering for them. It's uncomfortable and it works. It teaches tolerance for the anxious pause.

Rephrase to reduce pressure: Swap yes/no questions for a choice (“puppy sticker or star sticker?”) is easier to answer than an open question, and still counts as real progress.

Labelled praise: “Great job telling me you want juice” beats “good job”: specific praise reinforces the exact behaviour you want repeated.

Small group before whole class: Many children who can't speak in front of 25 peers can manage in front of 3, so use smaller settings as a genuine stepping stone, not a permanent accommodation.

3

Tweens & Teens (11+)

By this age, a young person has often built years of habit around silence, and progress needs to be their goal, not just the adults' around them.

Involve them in the plan: Ask what feels possible rather than deciding for them: autonomy matters more with age, and a goal they helped set is one they're more likely to work toward.

Rethink participation, not just speech: Recorded responses, written answers, or a device can demonstrate real understanding without demanding speech before the young person is ready.

Keep the adults quietly aligned: Discreet, consistent support across subjects and settings matters more than any single classroom strategy. A young person shouldn't have to renegotiate the same accommodation with every teacher.

Don't mistake a plateau for failure: Progress can slow around adolescence without treatment. That's a reason to stay engaged, not a sign that support has stopped being worthwhile.

The Language Toolkit: One Set of Strategies, Every Age

These five strategies work at every age band above, which is exactly the point: consistency between home and school (and across a young person's whole day) is what makes them effective, more than any single clever technique.

- Comment, don't question: “that's a red truck” invites a response without demanding one, unlike “what colour is the truck?”
- Pause and wait: resist the urge to fill silence; a genuine pause gives space for a response to emerge
- Expansion: take whatever the child offers, even a single word or gesture, and build one small piece onto it
- Parallel talk: describe what the child is doing in the moment, without requiring anything back
- Labelled, specific praise: name exactly what worked, so the child knows precisely what to repeat

A Realistic Practice Rhythm

The ideal is frequent, brief practice, something like fifteen minutes, three times a week. The reality, especially in a classroom, is that some weeks that becomes one ten-minute session, because that's genuinely all the time available. That's not a failure of the plan. Consistency that survives a busy term matters more than an ideal protocol that quietly stops happening after week two. Small, sustainable practice beats an ambitious plan nobody can keep up.

How This Ties to BRIDGE

	BRIDGE step	Applied to selective mutism
B	Behaviour Has a Reason	Silence is anxiety, not defiance or shyness. The starting lens has to be right before any strategy will work.
R	Record the Pattern	Where, with whom, and in what settings does speech happen or not, the same anxiety hierarchy (people / location / activity) clinicians use.
I	Identify One Shared Goal	One small, agreed next step, not “speak normally,” but the specific next rung on the ladder.
D	Design Matching Strategies	The same technique (comment, pause, expand) used identically at home and at school.
G	Give Consistent Language	The language toolkit above, used the same way by every adult in the child's day.

	BRIDGE step	Applied to selective mutism
E	Evaluate & Adjust Together	Small, regular check-ins on what's working. Progress in SM is measured in inches, and inches are worth noticing.

Filling the Gap

Selective mutism sits in the same space as much of neurodivergent support in Australia right now: long waits for a specialist assessment, and very little structured guidance offered in the meantime. You don't need a diagnosis or a referral in hand to start using the strategies in this guide, at home or in the classroom, today.

References

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